APEXCARE INDIA EMPLOYEE HEALTH BENEFIT PROGRAMME (AIEHBP)
FRAUD CONTROLS, PRIVACY, GRIEVANCE AND AUDIT STANDARD

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Primary audience: Internal compliance, investigation, privacy, grievance and audit teams

FICTIONAL TRAINING DOCUMENT - NOT AN INSURANCE CONTRACT. This document is a synthetic example created for software development, retrieval testing, and claims-workflow demonstrations. A real insurer, employer, TPA, legal team, medical team, and compliance function must review and approve all wording, benefits, timelines, forms, and regulatory references before operational use.

Document-control rule: cite the document code, section, clause, page, and version in every decision.
Interpretation rule: the signed policy schedule and endorsements override this demonstration text.

1. FRAUD, WASTE AND ABUSE PRINCIPLES

Controls must distinguish anomaly, error, abuse and proven fraud.

GOV-1.1 Red flag is not proof

A red flag initiates proportionate verification. It must not automatically stop emergency care, label a member fraudulent or produce final denial.

GOV-1.2 Potential member indicators

Possible indicators include altered invoices, duplicate claims, identity inconsistency and undisclosed reimbursement. Each may also have an innocent explanation.

GOV-1.3 Potential provider indicators

Possible indicators include impossible service patterns, unbundling, phantom billing, excessive stays, repeated identical notes and unusual referral concentration.

GOV-1.4 Evidence preservation

Preserve original files, hashes, metadata, communications, verification responses and investigator notes with controlled access.

GOV-1.5 Authorized outcome

Only authorized investigation and claims personnel may conclude fraud-related action, following contract and applicable due process.

2. VERIFICATION METHODS

Use the least intrusive reliable method appropriate to risk.

GOV-2.1 Document verification
Confirm invoice number, provider issuance, payment, admission and clinical records through approved channels.

GOV-2.2 Member contact
Use verified contact details, avoid disclosing diagnosis unnecessarily and document consent or authority.

GOV-2.3 Provider contact
Request only relevant records, authenticate the requester and record response provenance.

GOV-2.4 Analytics
Network and anomaly scores prioritize review; they do not determine guilt or coverage.

GOV-2.5 Clinical review
Medical plausibility questions must be assessed by qualified reviewers rather than investigators or language models alone.

3. PRIVACY AND DATA MINIMIZATION

Health and identity information requires heightened protection.

GOV-3.1 Purpose limitation
Collect and use claim data only for authorized insurance operations, legal duties, quality, fraud control and explicitly approved secondary purposes.

GOV-3.2 Minimum necessary
Retrieve and expose only fields and documents required for the task. Mask unrelated diagnoses, family data, account numbers and identifiers where practical.

GOV-3.3 Access control
Enforce role-based access, strong authentication, least privilege, periodic recertification and immediate revocation on role change.

GOV-3.4 Encryption and transfer
Protect stored and transmitted data using approved encryption and controlled transfer channels.

GOV-3.5 Model handling
Do not send claim data to an external model unless contractual, privacy, security, residency and retention controls are approved.

4. RETENTION AND DELETION

Production schedules must be approved against current legal and contractual requirements.

GOV-4.1 Retention schedule
Assign records a category, owner, legal basis, retention period and disposal method. This synthetic document does not prescribe a statutory period.

GOV-4.2 Legal hold
Suspend normal deletion for records subject to litigation, investigation, audit or regulatory hold.

GOV-4.3 Derived data
Embeddings, extracted fields, prompts, logs and model outputs are claim data and follow equivalent governance.

GOV-4.4 Secure disposal
Delete or destroy data verifiably across primary, cache, backup and vendor systems according to approved procedure.

5. GRIEVANCE AND REVIEW

Members need accessible review of adverse decisions.

GOV-5.1 Grievance intake
Accept grievances through approved digital, written and assisted channels. Record receipt, issue, requested remedy and communication needs.

GOV-5.2 Independent review
Where practical, a reviewer not responsible for the original adverse decision reassesses evidence, policy scope, clauses and calculation.

GOV-5.3 Additional evidence
Allow relevant new evidence and explain whether it changes the decision.

GOV-5.4 Response
Provide outcome, reasons, cited clauses, calculation, further escalation route and applicable external review information.

GOV-5.5 Timelines
Configure acknowledgment and resolution timelines from the current contract and regulatory requirements; do not hard-code an outdated legal period.

6. AUDIT TRAIL REQUIREMENTS

A complete trail permits reconstruction without relying on memory.

GOV-6.1 Input provenance
Record uploader, channel, receipt time, original filename, hash and transformation history.

GOV-6.2 Policy provenance
Record corpus version, policy ID, effective version, metadata filters, retrieved chunks, rankings and cited clauses.

GOV-6.3 Model provenance
Record model and prompt version, parameters, outputs, confidence and human changes without exposing secret credentials.

GOV-6.4 Decision provenance
Record reviewer, authority, reason codes, calculation, approval chain, communication and payment reference.

GOV-6.5 Immutability
Protect material audit records from unauthorized alteration and log permitted corrections as new events.

7. MODEL AND RETRIEVAL GOVERNANCE

Retrieval systems are controlled decision-support components.

GOV-7.1 Pre-deployment validation
Test representative, rare, adverse, multilingual, OCR-degraded, negated and conflicting-policy cases.

GOV-7.2 Gold set
Maintain reviewed questions linking claims to authoritative clauses and expected decision factors.

GOV-7.3 Change control
Re-indexing, model changes, chunking changes, policy updates and prompt changes require versioned testing and approval.

GOV-7.4 Monitoring
Monitor drift, retrieval misses, unsupported citations, policy mixing, latency, cost and decision disparities.

GOV-7.5 Kill switch
Provide a tested mechanism to disable automated recommendation and continue controlled manual processing.

8. INCIDENT RESPONSE

Security and decision incidents require coordinated action.

GOV-8.1 Incident types

Include unauthorized disclosure, corrupted index, wrong policy version, systematic false rejection, prompt injection and payment manipulation.

GOV-8.2 Containment

Restrict access, stop affected automation, preserve evidence and identify impacted claims and members.

GOV-8.3 Correction

Reprocess affected claims using a validated version and provide remediation and notification as required.

GOV-8.4 Learning

Complete root-cause analysis, corrective controls, regression tests and accountable closure.

9. PRODUCTION ADOPTION CHECKLIST

A company must complete these items before treating the pack as operational.

GOV-9.1 Legal approval
Replace fictional terms with approved policy wording and current jurisdiction-specific requirements.

GOV-9.2 Actuarial and underwriting approval
Validate benefits, limits, waiting periods, pricing assumptions and risk acceptance.

GOV-9.3 Clinical approval
Validate necessity rules, clinical pathways, modern treatments and review thresholds.

GOV-9.4 Privacy and security approval
Complete data-flow mapping, vendor assessment, threat modeling and access design.

GOV-9.5 Operational approval
Validate forms, provider contracts, payment controls, training, service levels and business continuity.

GOV-9.6 Explicit prohibition
Do not present this synthetic pack to a member as an issued policy or rely on it for a real claim decision.